CASE 3

Ms. Sophie Bennett — 34F · RRMS | Post-relapse (optic neuritis) · Fatigue · Leg heaviness >200 m · Bilateral foot paraesthesia · Balance impairment when fatigued · Fear of exercise-triggered relapse

Target: 70%+ | Neurological Physiotherapy

■ PROBLEM LIST

- Problems Identified
- Physical Impairments
- Primary central MS fatigue
- Bilateral lower limb sensory impairment (paraesthesia — feet)
- Balance impairment, worse with fatigue and heat
- Functional lower limb weakness / leg heaviness on exertion
- Likely residual visual deficit from optic neuritis
- Functional Activity Limitations
- Walking limited to <200 m before symptom onset
- Reduced confidence and safety with community ambulation
- Fall risk (balance compromised under fatigue loading)
- Participation Restrictions

■ RED FLAGS

- New or worsening bladder / bowel dysfunction
- New neurological deficit within the past 4–6 weeks (possible active relapse)
- Lhermitte's sign (new onset)
- New or rapidly worsening visual loss
- Severe or persistent depression / suicidal ideation

■ PATHOPHYSIOLOGY

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■ DIFFERENTIAL DIAGNOSES

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■ OBJECTIVE ASSESSMENTS & HOW TO PERFORM

Berg Balance Scale (BBS) Functional Balance

Test 1: Berg Balance Scale (BBS) Functional Balance What it measures: Static and dynamic balance across 14 functional tasks, scored 0–4 each (total 0–56). A score below 45 indicates significant fall risk in the community-dwelling MS population. How to perform: Set up a standard c

Romberg Test and Tandem Romberg Sensory Integration

Test 2: Romberg Test and Tandem Romberg Sensory Integration What it measures: Differentiates cerebellar ataxia (unstable with eyes open) from sensory/proprioceptive ataxia (stable with eyes open, markedly worse with eyes closed). Identifies the dominant source of balance impairme

Timed Up and Go (TUG) — Single and Dual Task Functional Mobility

Test 3: Timed Up and Go (TUG) — Single and Dual Task Functional Mobility What it measures: Functional mobility, dynamic balance, and fall risk. Normative value for community-dwelling adults: <12 seconds. In MS, >12 s indicates fall risk; the dual-task version assesses cognitive-m

■ TREATMENT PLAN

■ 1. Address Fear of Exercise — Education First

■ 2. Fatigue Management and Pacing

■ 3. Balance Retraining — Progressive Sensory Challenge